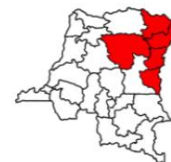




EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°095/MVEBDB/08/17/2026

Reporting date: August 17, 2026

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HIGHLIGHTS (last 24 hours)

In the last 24 hours, 84 new confirmed cases of Ebola Virus Disease, including 29 community cases, have been reported. (EVD) cases were recorded in the provinces of Ituri (52 cases), North Kivu (26 cases) and Haut Uélé (6). No new health zones were affected.

ACCUMULATIVE CASE 5,105	ACCUMULATIVE DEATH CONFIRMED 2,420	LETHALITY 47.4%	PATIENTS IN ISOLATION/CTE 782	ACCUMULATIVE HEALED 1091	TRACKING RATES CONTACTS (Of the Day) 84.1%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



55 out of 151 health zones affected (36.4%)



Summary of key points from the last 24 hours

In the past 24 hours, 84 new confirmed cases and 29 community deaths from Ebola Virus Disease (EVD) have been reported in the provinces of **Ituri (52 cases), North Kivu (26) and Haut Uélé (6)**.

At the end of the day on August 17, 2026, **30 patients** were declared recovered after receiving two negative results for Ebola Virus Disease (EVD) (27 in Ituri and 3 in Haut-Uélé). However, 13 confirmed deaths within Ebola Treatment Centers (ETCs) were recorded, including 12 in Ituri and 1 in Haut-Uélé.

Thus, the Democratic Republic of Congo has reached a cumulative total of 5,105 confirmed cases, including 2,420 deaths, representing a case fatality rate of 47.4%. Ituri remains the epicenter of the epidemic, accounting for 84.4% of cumulative confirmed cases and nearly two-thirds (61.9%) of new cases reported in the last 24 hours.

No new health zones were affected. In total, 55 of the 151 health zones in the six provinces have been affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas Uélé (1/11).

Distribution of confirmed cases and deaths by affected province

(As of August 17, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4309	1915	44.4%	28/36 (77.8%)	52
North Kivu	633	431	68.1%	12/34 (35.3%)	26
Haut-Uélé	145	65	44.8%	6/13 (46.1%)	6
Tshopo	14	7	50.0%	7/23 (30.4%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Bas Uélé	1	1	100.0%	1/11 (9.1%)	0
Total	5,105	2,420	47.4%	55/151 (36.4%)	84

Confirmed cases and deaths by province and health zone (situation as of August 17, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4309	1915	44.4%	52	25	12	37
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	8	2	25.0%	1	0		0
Aungba	11	5	45.5%				0
Bamboo	112	26	23.2%	3	1		1
Boga	1	1	100.0%				0
Bunia	1194	372	31.2%	20	14		14
Damascus	25	9	36.0%				0
Drodro	13	6	46.2%				0
Fataki	71	30	42.3%	2	0		0
Gethy	5	2	40.0%	1	1		1
Kambala	2	0	0.0%				0
Kilo	31	12	38.7%				0
Komanda	51	35	68.6%	3	3		3
Lita	183	104	56.8%	2	2		2
Logo	11	5	45.5%				0
Lolwa	15	4	26.7%				0
Mahagi	4	1	25.0%				0
Mambasa	14	7	50.0%				0
Mandima	23	14	60.9%				0
Mangala	165	92	55.8%	3	0		0

Mongbwalu	591	282	47.7%	1	0		0
Nia-Nia	175	103	58.9%	5	3		3
Nizi	543	240	44.2%	4	0		0
Nyankunde	121	35	28.9%				0
Rimba	9	4	44.4%				0
Rwampara	863	332	38.5%	7	1		1
Tchomia	50	25	50.0%				0
To be ventilated	N / A	160	N / A			12	12
North Kivu	633	431	68.1%	26	3	0	3
Blessed	96	71	74.0%	3	1		1
Butembo	118	96	81.4%	6	1		1
Goma	1	0	0.0%				0
Kalunguta	13	6	46.2%				0
Katwa	306	199	65.0%	15	1		1
Kyondo	15	12	80.0%				0
Lubero	1	1	100.0%				0
Mabalako	2	1	50.0%				0
Masereka	7	4	57.1%				0
Musienene	63	35	55.6%	1	0		0
Oicha	6	4	66.7%	1	0		0
Vuhovi	5	2	40.0%				0
Haut-Uélé	145	65	44.8%	6	1	1	2
Boma Mangbetu	23	8	34.8%	1	0		0
Gombari	1	1	100.0%				0
Isiro	47	22	46.8%	5	1	1	2
Pawa	20	14	70.0%				0
Rungu	1	0	0.0%				0
Wamba	53	20	37.7%				0
Tshopo	14	7	50.0%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	3	2	66.7%				0
Lubunga	1	0	0.0%				0
Makiso- Kisangani	5	4	80.0%				0
Mangobo	2	1	50.0%				0
Tshopo	1	0	0.0%				
Wanie-Rukula	1	0	0.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	1	1	100.0%	0	0	0	0
Buta	1	1	100.0%				0
Total	5105	2420	47.4%	84	29	13	42

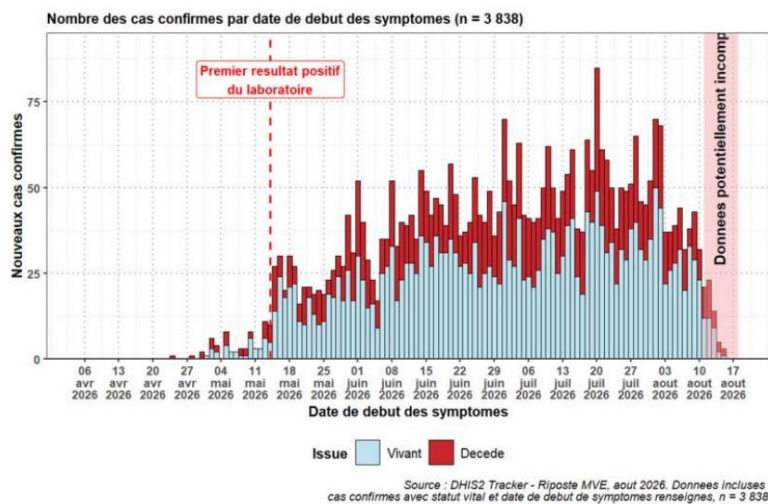
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable

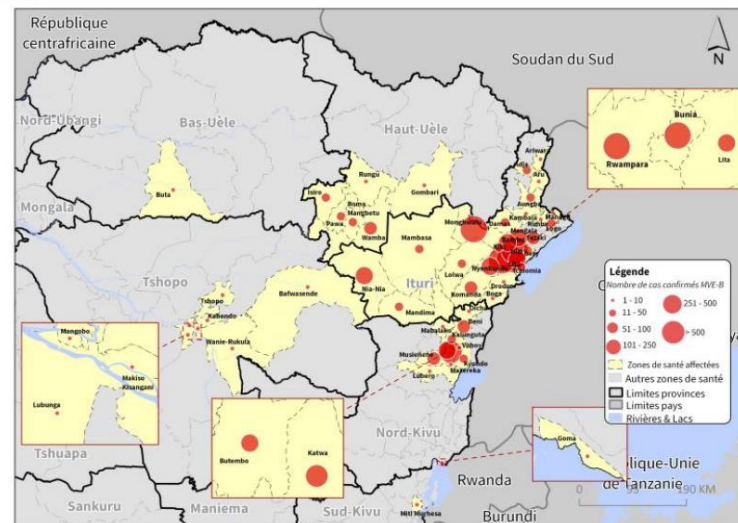
Status of alerts notified by province, as of August 17, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	18	1	19	16	1	2	0	17	13
Bas Uélé	2	1	3	2	1	0	0	3	2
Ituri	646	80	726	159	78	75	2	135	47
North Kivu	783	64	847	82	46	698	18	127	51
South Kivu	10	0	10	5	0	5	0	5	5
Tshopo	49	0	49	4	0	45	0	4	0
Grand Total 1508		146	1,654	268	126	825	20	291	118

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Site evaluation for the future CTE at ITAV; evaluation visit of care at the Kitumba CTE with the Ugandan team and meetings with the sub-coordinations of Kyondo and Katwa.
- **Ituri/Tshopo** : continued evaluation of the technical platform of the CTEs using the 5S tool, The first count is expected on August 18, 2026.
- **Bas-Uélé**: Meeting with civil society and the Minister of Health on resistance and launch of the MEURI clinical trial. Haut-Uélé: Awareness-raising for management teams from the 13 health zones gathered in Isiro.

1.2. Epidemiological surveillance

1.2.1. Main Actions

At the end of the day on August 17, 2026, 1,654 alerts were recorded, including 1,239 (74.9 % were verified in the six provinces. Of these alerts, 394 (31.8%) were validated as suspected cases and 291 (73.9%) were investigated, of which 105 were transferred to CT/CTE.

The proportion of contacts monitored in the last 24 hours has risen to 84.1% (20 337 views out of 24,332 to follow), above the 85% threshold: Ituri 93.0% (13,171/15,160), Tshopo 100% (483/483) and North Kivu 80.1% (5,987/7,473). Haut-Uélé 64.3% (661/1 028) and Bas Uélé 18.6% (35/188).

1.2.2. Challenges

Low reporting of alerts and listing and monitoring of contacts around confirmed cases.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Six (6) alerts were detected at PoE/PoC during the last 24 hours: three in Ituri (PoC Mudzibala, ZS of Bunia), all validated and referred to the CTE (3/3), two in North Kivu (PoC of Kasitu and Kahongo) one of which was transferred (1/2), and a lifeless body intercepted at the PK6 Saplast barrier (ZS of Isiro, Haut-Uélé), sampled and tested positive (swab 1/1).

Monitoring of indicators at PoE/PoC by province, as of August 17, 2026

	Ituri	North Kivu	Haut-Uélé	South Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100%	100%	28.9%	ND	76.7%
Number of people who have switched to PoE/ PoC	141,637	89,357	5,780	10,896	247,670
% of travelers screened	97.5%	97.3%	99.0%	98.5%	97.5%
% of travelers who washed their hands	97.4%	97.3%	99.3%	98.5%	97.5%
% of travelers made aware	99.5%	97.3%	99.2%	100%	98.7%
Number of alerts notified	3	2	1	0	6
Number of validated alerts alive	3	2	0	0	5
Number of bodies recovered today	0	0	1	0	1
Number of problematic contacts intercepted today	0	0	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100%	50%	N / A	N / A	80.0%
% of lifeless bodies swabed	N / A	N / A	100%	N / A	100%

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: in Haut-Uélé, only 11 entry points are activated out of the 38 priority points (28.9%) and 5 out of 11 have reported (45.5% completeness), due to a lack of training, supervision and logistical and financial support.

- Constraints affecting the functionality of PoE/PoC: non-payment of service providers in North Kivu and a strike in Haut-Uélé which deprived six PoCs (Dusu, Apodo, Aungba, PK6 Route Niangara, Gossamu and Magambe) of reporting, aggravated by the disruption of inputs; 2,383 refusals of screening in North Kivu (2.73%) and 58 in Haut-Uélé (1%), motivated by denial of the disease, fear of contamination by water and fear of internment at the CTE; non-compliance with health requirements at PoE/PoC by some officials.

1.4. Laboratory

1.4.1. Main actions

Ituri recorded 52 new positive results (27 living and 25 deaths) out of 287 New samples received and analyzed (Positivity rate: 18.1%). In addition, 38 Tests came back positive out of 90 samples analyzed. North Kivu confirmed 26 new cases, including 3 deaths, out of 192 samples analyzed (positivity rate: 13.5%), while Haut-Uélé confirmed 6 cases (5 living and 1 deceased). Conversely, 6 samples from Bas-Uélé and all 2 from South Kivu came back negative.

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- **In Ituri**, 7 rings were opened out of 152 expected (4.6%) and 44 households decontaminated out of 47 identified, for 12 allocations of PCI/EHA kits; 85 death alerts resulted in 58 bodies collected by swab, 15 failures and 73 EDS carried out.
- **In North Kivu**, 34 rings were opened, 35 households and 7 FOSA decontaminated; 47 bodies were collected without failure for 44 EDS carried out, the deaths being concentrated in Beni (36.1%) and Katwa (31.9%).
- **In Haut-Uélé**, 10 rings were opened (4 in Isiro, 4 in Wamba and 2 in Pawa) and 9 bodies samples taken for 5 EDS carried out; the Wamba health zone recorded 6 deaths in a single day, a first.
- **In Bas-Uélé**, a ring was opened in Buta, but no EDS could be carried out, the remains having been removed by relatives.

1.5.2. Challenges

- Weak implementation of IPC standards: limited opening of rings in Ituri (7 out of 152) and incomplete decontamination (44 households out of 47 expected) due to the strike of decontamination workers, insufficient teams, insecurity and addresses not found; completeness of IPC reporting limited to 9 out of 14 health zones in Ituri and to 7 out of 12 in North Kivu, three health zones in Haut-Uélé (Boma Mangbetu, Gombari and Rungu) having transmitted no data; claim for payment of decontamination teams in Butembo, Beni, Katwa and Musienene for June and July 2026; in Haut-Uélé, a lifeless body was taken by the family in Isiro with total refusal to receive the investigators.

1.6. Continuity of care

1.6.1. Main actions

In Ituri, 79 new admissions, all suspected cases, and 11 readmissions were recorded, with 120 discharges, including 27 recoveries. Thus, 470 patients are hospitalized (172 confirmed and 298 suspected) in 996 beds, representing an occupancy rate of 47.2%, and the provincial total of recoveries has reached 1,000. The treatment centers in Bambu and Nizi (confirmed cases) as well as in Fataki, Kilo, Logo, Nia-Nia, and Kigonze (suspected cases) are at capacity. Thirty-four out of 35 ambulances are available, with 16 in use and 24 successful ambulance runs.

- **In North Kivu**, 20 admissions were recorded (2 confirmed at the Katwa Ebola Treatment Center and 18 suspected cases) with 12 discharges, including 1 suspected death. Thus, 181 patients are hospitalized (34 confirmed and 147 suspected cases) in 206 beds, representing an 87.9% occupancy rate, with suspected cases reaching 103.5%. Seven ambulances are available and in use.

In Haut-Uélé, seven Ebola Treatment Centers (ETCs) are operational but insufficiently equipped, with only one meeting standards (Watsa). Three new admissions and three recoveries (one in Rungu and two in Boma Mangbetu) were recorded. Thus, 49 patients are hospitalized (39 confirmed and 10 suspected) out of 78 beds, representing a 62.8% occupancy rate, with critical saturation of confirmed beds in Wamba (400%) and Isiro (216%). In Bas-Uélé, one suspected case was admitted to the temporary ETC in Buta. In South Kivu, 14 suspected cases remain hospitalized, and the province has gone 68 days without a new confirmed case. Tshopo has not reported any new cases.

1.6.2. Challenges

Insufficient availability of referral resources (1 ambulance out of service in Bambu in Ituri, only 7 ambulances in North Kivu and no ambulance dedicated to the response in Bas-Uélé) and low capacity of some CTE/CT: CTE of Bambu (confirmed and suspected) and of Nizi (confirmed) as well as Fataki, Kilo, Logo, Nia-Nia and Kigonze (suspected) saturated in Ituri; CTE of Kitatumba and Katwa and isolation of Musienene saturated in North Kivu (103.5% occupancy of suspected beds); confirmed beds saturated in Wamba (400%) and in Isiro (216%) in Haut-Uélé, where the CTEs lack water, electricity and sanitation; limited capacity and water supply at the CTE of Buta. A pregnant woman had a miscarriage at the Pawa CTE and comorbidities associated with Ebola virus disease remain insufficiently managed.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, 112 new contacts were identified around the day's cases. Five testing rings were opened and 10 households were decontaminated in Rwampara; 630 people were reached through awareness campaigns, 22 radio programs and 6 radio spots were broadcast, and 42 IEC (Information, Education, and Communication) materials were distributed.

In Ituri, the SMSPS pillar covered 19 of the 41 new confirmed cases (46.3%) and 27 of the 89 new suspected cases (30.3%), and 17 of the 27 recoveries have been reintegrated into the community. In Bas-Uélé, a meeting was held with civil society representatives and the Minister of Health. Haut-Uélé and Tshopo did not submit any community data.

- **In North Kivu**, 81 household heads in 11 health zones and 44 contacts were identified in Butsili; 385 people received psychoeducation, 18 separated or orphaned children received support, and 12 rumors were addressed. Thirty journalists from 13 radio stations were briefed in Beni.

1.1.1. Challenges

- Persistent community challenges: 7 unresolved community incidents in the North Kivu, all in the Musienene Health Zone; resistance to the Health and Social Development (HSD) in Ituri (Nyankunde, Mongbwalu/Lodjo Health Zone, Komanda, Aru, and Nia-Nia) and the unsafe removal of a body in Bas-Uélé, which prevented the HSD; poor organization of home visits by community health workers in Ituri (Nizi and Nyankunde Health Zones) due to non-payment; need for communication materials (posters, leaflets, banners). Only 8 of the 28 affected health zones in Ituri and 2 in North Kivu reported community concerns, representing 19% compared to 23% in the previous period.

1.8. Logistics

1.8.1. Main actions

In Ituri, 1,290 kg of PCI and PECH kits were delivered to the Fataki Health Zone and 244 kg of PCI-WASH kits to the PECH pillar; work on the Nia-Nia CTE continues and the new Mongbwalu CTE building is operational with 60 beds.

In North Kivu, the construction of the triage facility at the Vuhovi General Referral Hospital and the rehabilitation of the Musienene Treatment Center are continuing; PCI inputs have been delivered to the Kalunguta and Mabalako Health Zones and nutritional inputs deployed to the Kibua Health Zone.

In Bas-Uélé, five motorcycles were allocated to the Buta health zone, and a tent was erected while awaiting a fully compliant treatment center. In Haut-Uélé, the University and Eastern clinics (Isiro health zone) received infection prevention and control supplies. Tshopo did not report.

1.8.2. Challenges

1.9. Ongoing logistical needs to support the expansion of the response: completion of the extensions to the Nizi, ISTM, and Bambu treatment centers in Ituri; needs for 4 glove boxes (Fataki, Bambu-mine, Lita, and Rwampara health zones), cryotubes, and cryoboxes; shortages of supplies, PPE, and water in all activated PoE/PoC sites in Haut-Uélé and lack of transportation for supervisory and EDS (Emergency Response Teams); deployment of IPC (Infection Prevention and Control) supplies to the 7 zones in western and northern Bas-Uélé and transport of samples to the Rad-One site; lack of a hearse in all health zones of North Kivu and stockouts of PPE in the treatment centers and treatment centers

1.10. Security

1.10.1. Main actions

- In North Kivu, security teams accompanied four EDS (security, swab, and burial) operations, all of which were completed. In Haut-Uélé, the peacekeeping force maintained public order at the Pimpester Hospital and the Saplast checkpoint and ensured the return of the intercepted body to the Isiro Transfusion Center; the situation remains stable in all 13 security zones except for part of Gombari.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions

- Awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) continued in the affected provinces. No daily data was submitted by the pillar for the period under review; activities reported in the previous cycle are continuing.

1.1.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse is still insufficient in all areas concerned by the response.

KEY MESSAGES

- Transmission of Ebola virus disease remains active, with 84 new cases including 29 community deaths and 13 intra-CTE recorded in the last 24 hours, bringing the national total to 5,105 confirmed cases and 2,420 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 84.4% of cumulative confirmed cases and 61.9% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows 73.9% of suspected cases investigated while the follow-up rate (84.1%) is below the threshold of 85.0%.

• Strengthening priority interventions

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of 64 support teams.



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